NOVEXIB 200 MG CAPSULES
Celecoxib

COMPOSITION
Each capsule contains 200 mg of Celecoxib.
Excipients: Croscarmellose sodium, Lactose monohydrate, Povidone, Magnesium stearate, Sodium lauryl sulphate, Purified water.
Description: Hard capsule color aqua blue (cap) and aqua blue (body), white granules.

PHARMACODYNAMICS
Pharmacotherapeutic group: Non-steroidal anti-inflammatory and antirheumatic drugs, NSAIDs, Coxibs ATC Code : M01AH01

Mechanism of action
Celecoxib is an oral, selective, cyclooxygenase-2 (COX-2) inhibitor within the clinical dose range (200-400 mg daily).

Pharmacodynamic effects
Cyclooxygenase is responsible for generation of prostaglandins. Two isoforms, COX-1 and COX-2, have been identified. COX-2 is the isoform of the enzyme that has been shown to be induced by pro-inflammatory stimuli and has been postulated to be primarily responsible for the synthesis of prostanoid mediators of pain, inflammation, and fever. COX-2 is also involved in ovulation, implantation and closure of the ductus arteriosus, regulation of renal function, and central nervous system functions (fever induction, pain perception and cognitive function). It may also play a role in ulcer healing. COX-2 has been identified in tissue around gastric ulcers in humans but its relevance to ulcer healing has not been established.
The difference in antiplatelet activity between some COX-1 inhibiting NSAIDs and COX-2 selective inhibitors may be of clinical significance in patients at risk of thrombo-embolic reactions. COX-2 selective inhibitors reduce the formation of systemic (and therefore possibly endothelial) prostacyclin without affecting platelet thromboxane.
Celecoxib is a diaryl-substituted pyrazole, chemically similar to other non-arylamine sulfonamides (e.g. thiazides, furosemide) but differs from arylamine sulfonamides (e.g. sulfamethoxizole and other sulfonamide antibiotics).
A dose-dependent effect on TxB2 formation has been observed after high doses of celecoxib. However, celecoxib may have no effect on platelet aggregation and bleeding time on small mulitple doses.

PHARMACOKINETICS
Absorption
Celecoxib is well absorbed reaching peak plasma concentrations after approximately 2-3 hours. Dosing with food (high fat meal) delays absorption of celecoxib by about 1 hour resulting in a Tmax of about 4 hours and increases bioavailability by about 20%.
The overall systemic exposure (AUC) of celecoxib was equivalent when celecoxib was administered as intact capsule or capsule contents sprinkled on applesauce. There were no significant alterations in Cmax, Tmax or T1/2 after administration of capsule contents on applesauce.

Distribution
Plasma protein binding is about 97 % at therapeutic plasma concentrations and the medicinal product is not preferentially bound to erythrocytes.

Biotransformation
Celecoxib metabolism is primarily mediated via cytochrome P450 2C9. Three metabolites, inactive as COX-1 or COX-2 inhibitors, have been identified in human plasma i.e., a primary alcohol, the corresponding carboxylic acid and its glucuronide conjugate. Cytochrome P450 2C9 activity is reduced in individuals with genetic polymorphisms that lead to reduced enzyme activity, such as those homozygous for the CYP2C9*3 polymorphism.

Elimination
Celecoxib is mainly eliminated by metabolism. Less than 1 % of the dose is excreted unchanged in urine. The intersubject variability in the exposure of celecoxib is about 10-fold. Celecoxib exhibits dose- and time-independent pharmacokinetics in the therapeutic dose range. Elimination half-life is 8-12 hours. Steady state plasma concentrations are reached within 5 days of treatment.

Food Effects
Dosing with food (high fat metal) delays absorption of celecoxib in a Tmax of about 4 hours and increases bioavailability by about 20%. The overall systemic exposure (AUC) of celecoxib was equivalent when celecoxib was administered as intact capsule or capsule contents sprinkled on applesauce. There were no significant alterations in Cmax, Tmax or T1/2 after administration of capsule contents on applesauce.

Special populations
Elderly
In the population >65 years there is a one and a half to two-fold increase in mean Cmax and AUC for celecoxib. This is a predominantly weight-related rather than age-related change, celecoxib levels being higher in lower weight individuals and consequently higher in the elderly population who are generally of lower mean weight than the younger population. Therefore, elderly females tend to have higher drug plasma concentrations than elderly males. No dosage adjustment is generally necessary. However, for elderly patients with a lower than average body weight (<50 kg), initiate therapy at the lowest recommended dose.

Race
Approximately 40% higher AUC of celecoxib in the Black population compared to Caucasians.

Hepatic impairment
Plasma concentrations of celecoxib in patients with mild hepatic impairment (Child-Pugh Class A) are not significantly different from those of age and sex matched controls. In patients with moderate hepatic impairment (Child-Pugh Class B) celecoxib plasma concentrations are about doubled up.

Renal impairment
In elderly with age-related reductions in glomerular filtration rate (GFR) (mean GFR >65 ml/min/1.73 m^2) and in patients with chronic stable renal insufficiency (GFR 35-60 ml/min/1.73 m^2) celecoxib pharmacokinetics was comparable to those seen in patients with normal renal function. No significant relationship between serum creatinine (or creatinine clearance) and celecoxib clearance. Severe renal insufficiency would not be expected to alter clearance of celecoxib since the main route of elimination is via hepatic metabolism to inactive metabolites.

Renal Effects
The relative roles of COX-1 and COX-2 in renal physiology are not completely understood. Celecoxib reduces the urinary excretion of PGE2 and 6-keto-PGF1μ (a prostacyclin metabolite) but leaves serum thromboxane B2 (TXB2) and urinary excretion of 11-dehydro-TXB2, a thromboxane metabolite (both COX-1 products) unaffected. Celecoxib produces no decreases in GFR in the elderly or those with chronic renal insufficiency.

INDICATION
- For the management of acute pain in adults and for the treatment of primary dysmenorrhoea.
- Relief of the acute and chronic pain and inflammation of rheumatoid arthritis and osteoarthritis.
- Relief of signs and symptoms of ankylosing spondylitis.
- For the management of low back pain.

RECOMMENDED DOSAGE
Celecoxib capsules can be taken with or without food. Given the association between cardiovascular risk and exposure to COX-2 inhibitors, doctors are advised to use the lowest effective dose for the shortest possible duration of treatment.

Adults
Symptomatic Treatment of Osteoarthritis (OA): The recommended dose of celecoxib is 200 mg administered as a single dose or as 100 mg twice per day
Symptomatic Treatment of Rheumatoid Arthritis (RA): The recommended dose of celecoxib is 100 mg or 200 mg twice per day.
Ankylosing Spondylitis (AS): The recommended dose of celecoxib is 200 mg administered as a single dose or 100 mg twice per day. Some patients may benefit from a total daily dose of 400 mg.
Management of Acute Pain: The recommended dose of celecoxib is 400 mg, initially, followed by an additional 200 mg dose, if needed on the first day. On subsequent days, the recommended dose is 200 mg twice daily, as needed.
Treatment of Primary Dysmenorrhoea: The recommended dose of celecoxib is 400mg, initially, followed by an additional 200 mg dose, if needed on the first day. On subsequent days, the recommended dose is 200 mg twice daily, as needed.
Low Back Pain (LBP): Usual dosage for adults is 100 mg of celecoxib orally twice daily, morning and evening after meal, or 200 mg once daily (100 mg and 200 mg only).
Elderly: No dosage adjustment is generally necessary. However, for elderly patients with a lower than average body weight (<50 kg), it is advisable to initiate therapy at the lowest recommended dose.
Hepatic impairment: No dosage adjustment is necessary in patients with mild hepatic impairment (Child-Pugh Class A). Introduce celecoxib at half the recommended dose in arthritis or pain patients with moderate hepatic impairment (Child-Pugh Class B). Patients with severe hepatic impairment (Child-Pugh Class C) have not been studied.
Renal impairment: No dosage adjustment is necessary in patients with mild or moderate renal impairment. There is no clinical experience in patients with severe renal impairment.
Children: Celecoxib is not indicated for use in children.
CYP2C9 Poor Metabolizers: Patients who are known, or suspected to be CYP2C9 poor metabolizers based on genotyping or previous history/experience with other CYP2C9 substrates should be administered celecoxib with caution as the risk of dose-dependent adverse effects is increased. Consider reducing the dose to half the lowest recommended dose

ROUTE OF ADMINISTRATION
Oral

CONTRAINDICATIONS
Celecoxib is contraindicated in:
- Patients with hypertension (high (high blood pressure) whose blood pressure is not under control)
- Patients with known hypersensitivity to celecoxib or any other ingredient of the product.
- Patients with known hypersensitivity to sulfonamides.
- Active peptic ulceration or gastrointestinal (GI) bleeding.
- In pregnancy and in women of childbearing potential unless using an effective method of contraception. The potential for human risk in pregnancy is unknown, but cannot be excluded.
- Breast-feeding.
- Severe hepatic dysfunction (serum albumin <25 g/l or Child-Pugh score ≥10).
- Patients with estimated creatinine clearance <30 ml/min.
- Inflammatory bowel disease.
- Congestive heart failure (NYHA II-IV).
- Patients who have experienced asthma, acute rhinitis, nasal polyps, angioneurotic oedema, urticaria or other allergic-type reactions after taking acetylsalicylic acid (aspirin) or other no-steroidal anti-inflammatory drugs (NSAIDs) including COX-2 inhibitors.
- Treatment of peri-operative pain in the setting of coronary artery bypass graft (CABG) surgery.
- Patients who have increased risk of cardiovascular disease (ischaemic heart disease and stroke)

WARNING AND PRECAUTIONS
WARNING
RISK OF GI ULCERATION, BLEEDING AND PERFORATION WITH NSAID
Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAID therapy. Although minor upper GI problems (e.g. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms.
Studies to date have not identified any subset of puberty not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious GI events and other risk factors associated with peptic ulcer disease (e.g. alcoholism, smoking, and corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events.
Patients with risk factors of heart disease, hypertension, hyperlipidaemia, diabetes mellitus, smoking patients and patients with peripheral arterial disease should only be treated with celecoxib after careful consideration.
Warning to prescriber when prescribing COX-2 Inhibitors to patients with risk factors of heart disease, hypertension (high blood pressure), hyperlipidemia, diabetes, smoking patient and patient with peripheral arterial disease.

Cardiovascular Effects
Cardiovascular Thrombotic Events:
Celecoxib may cause an increased risk of serious CV thrombotic events, myocardial infarction (MI), and stroke, which can be fatal. All NSAIDs may have a similar risk. This risk may increase with dose and duration of use. The relative increase of this risk appears to be similar in those with or without known CV disease or CV risk factors. However, patients with CV disease or CV risk factors may be at greater risk in terms of absolute incidence, due to their increased rate at baseline. To minimize the potential risk for an adverse CV event in patients treated with celecoxib, the lowest effective dose should be used for the shortest duration possible. Physicians and patients should remain alert for the development of such events, even in the absence of previous CV symptoms. Patients should be informed about the signs and symptoms of serious CV toxicity and the steps to take if they occur. Celecoxib is not a substitute for acetylsalicylic acid for prophylaxis of CV thrombo embolic diseases because of the lack of effect on platelet function. Because celecoxib does not inhibit platelet aggregation, anti-platelet therapies (e.g., acetylsalicylic acid) should not be discontinued.

Hypertension:
As with all NSAIDS, celecoxib can lead to the onset of new hypertension or worsening of pre-existing hypertension, either of which may contribute to the increased incidence of CV events. Patients taking angiotensin converting enzyme (ACE) inhibitors, thiazide diuretics or loop diuretics may have impaired response to these therapies when taking NSAIDs. NSAIDs, including celecoxib, should be used with caution in patients with hypertension. Blood pressure should be monitored closely during the initiation of therapy with celecoxib and throughout the course of therapy.

Fluid Retention and oedema:
As with other drugs known to inhibit prostaglandin synthesis, fluid retention and oedema have been observed in some patients taking celecoxib. Therefore, patients with pre-existing congestive heart failure (CHF) or hypertension should be closely monitored. Celecoxib should be used with caution in patients with compromised cardiac function, pre-existing o edema, or other conditions predisposing to, or worsened by, fluid retention including those taking diuretic treatment or otherwise at risk of hypovolemia.

Gastrointestinal (GI) effects
Upper and lower GI perforations, ulcers or bleeds have occurred in patients treated with celecoxib. These serious adverse events can occur at any time, with or without warning symptoms, in patients treated with Celecoxib. Patients most at risk of developing these types of GI complications with NSAIDs are the elderly, patients with CV disease, patients using concomitant aspirin, glucocorticoids, or other NSAIDs, patients using alcohol or patients with a prior history of, or active, GI disease, such as ulceration, GI bleeding or inflammatory conditions. Other factors that increase the risk of GI bleeding in patients treated with NSAIDs include longer duration of NSAID therapy; concomitant use of oral corticosteroids, aspirin, anticoagulants; or selective serotonin reuptake inhibitors (SSRIs); smoking; use of alcohol; older age; and poor general health status. Most spontaneous reports of fatal GI events have been in elderly or debilitated patients.

Strategies to Minimize the GI Risks in NSAID-treated patients:
- Use the lowest effective dosage for the shortest possible duration.
- Avoid administration of more than one NSAID at a time.
- Avoid use in patients at higher risk unless benefits are expected to outweigh the increased risk of bleeding. For such patients, as well as those with active GI bleeding, consider alternate therapies other than NSAIDs.
- Remain alert for signs and symptoms of GI ulceration and bleeding during NSAID therapy.
- If a serious GI adverse event is suspected, promptly initiate evaluation and treatment, and discontinue Celecoxib until a serious GI adverse event is ruled out.
- In the setting of concomitant use of low-dose aspirin for cardiac prophylaxis, monitor patients more closely for evidence of GI bleeding.

Renal Effects
NSAIDs including celecoxib may cause renal toxicity. Patients at greatest risk for renal toxicity are those with impaired renal function, heart failure, liver dysfunction, and the elderly. Such patients should be carefully monitored while receiving treatment with celecoxib.
Caution should be used when initiating treatment in patients with dehydration. It is advisable to rehydrate patients first and then start therapy with celecoxib.

Advanced Renal Disease
Renal function should be closely monitored in patients with advanced renal disease who are administered celecoxib.

Anaphylactoid Reactions
As with NSAIDs in general, anaphylactoid reactions have occurred in patients exposed to celecoxib.

Serious Skin Reactions
Serious skin reactions, some of them fatal, including exfoliative dermatitis, Stevens-Johnson syndrome, and toxic epidermal necrolysis, have been reported very rarely in association with the use of celecoxib. Patients appear to be at highest risk for these events early in the course of therapy, the onset of the event occurring in the majority of cases within the first month of treatment. Celecoxib should be discontinued at the first appearance of skin rash, mucosal lesions, or any other sign of hypersensitivity.

Hepatic Effects
Patients with severe hepatic impairment (Child-Pugh Class C) have not been studied. The use of celecoxib in patients with severe hepatic impairment is not recommended. Celecoxib should be used with caution when treating patients with moderate hepatic impairment (Child-Pugh Class B), and initiated at half the recommended dose.
Rare cases of severe hepatic reactions, including fulminant hepatitis (some with fatal outcome), liver necrosis, and hepatic failure (some with fatal outcome or requiring liver transplant), have been reported with celecoxib.
A patient with symptoms and/or signs of liver dysfunction, or in whom an abnormal liver function test has occurred, should be monitored carefully for evidence of the development of a more severe hepatic reaction while on therapy with celecoxib.

Use with Oral Anticoagulants
The concomitant use of NSAIDs with oral anticoagulants increases the risk of bleeding and should be given with caution. Oral anticoagulants include warfarin/coumarin-type and novel oral anticoagulants (e.g., apixaban, dabigatran, and rivaroxaban). In patients on concurrent therapy with warfarin or similar agents, serious bleeding events, some of them fatal, have been reported. Because increases in prothrombin time (INR) have been reported, anticoagulation/INR should be monitored in patients taking a warfarin/coumarin-type anticoagulant after initiating treatment with celecoxib or changing the dose.

General
By reducing inflammation, celecoxib may diminish the utility of diagnostic signs, such as fever, in detecting infections. The concomitant use of celecoxib and a non-aspirin NSAID should be avoided.

CYP 2D6 inhibition
Celecoxib inhibits CYP2D6. Although it is not a strong inhibitor of this enzyme, a dose reduction may be necessary for individually dose-titrated drugs that are metabolised by CYP2D6

INTERACTIONS WITH OTHER MEDICAMENTS
Pharmacodynamic interactions
Anticoagulants
Anticoagulant activity should be monitored particularly in the first few days after initiating or changing the dose of celecoxib in patients receiving warfarin or other anticoagulants since these patients have an increased risk of bleeding complications. Therefore, patients receiving oral anticoagulants should be closely monitored for their prothrombin time INR, particularly in the first few days when therapy with celecoxib is initiated or the dose of celecoxib is changed. Bleeding events in association with increases in prothrombin time have been reported, predominantly in the elderly, in patients receiving celecoxib concurrently with warfarin, some of them fatal.

Anti-hypertensives
NSAIDs may reduce the effect of anti-hypertensive medicinal products including ACE-inhibitors, angiotensin II receptor antagonists, diuretics and beta-blockers. As for NSAIDs, the risk of acute renal insufficiency, which is usually reversible, may be increased in some patients with compromised renal function (e.g. dehydrated patients, patients on diuretics, or elderly patients) when ACE-inhibitors, angiotensin II receptor antagonists, and/or diuretics are combined with NSAIDs, including celecoxib. Therefore, the combination should be administered with caution, especially in the elderly. Patients should be adequately hydrated and consideration should be given to monitoring of renal function after initiation of concomitant therapy, and periodically thereafter.

Ciclosporin and tacrolimus
Co-administration of NSAIDs and ciclosporin or tacrolimus may increase the nephrotoxic effect of ciclosporin or tacrolimus, respectively. Renal function should be monitored when celecoxib and any of these medicinal products are combined.

Acetylsalicylic acid
Celecoxib can be used with low-dose acetylsalicylic acid but is not a substitute for acetylsalicylic acid for CV prophylaxis. In the submitted studies, as with other NSAIDs, an increased risk of gastrointestinal ulceration or other gastrointestinal complications compared to use of celecoxib alone was shown for concomitant administration of low-dose acetylsalicylic acid

Pharmacokinetic interactions
Effects of celecoxib on other medicinal products
CYP2D6 inhibition
Celecoxib is an inhibitor of CYP2D6. The plasma concentrations of medicinal products that are substrates of this enzyme may be increased when celecoxib is used concomitantly. Examples of medicinal products which are metabolised by CYP2D6 are antidepressants (tricyclics and SSRIs), neuroleptics, anti-arrhythmic medicinal products, etc. The dose of individually dose-titrated CYP2D6 substrates may need to be reduced when treatment with celecoxib is initiated or increased if treatment with celecoxib is terminated. Concomitant administration of celecoxib 200 mg twice daily resulted in 2.6-fold and 1.5-fold increases in plasma concentrations of dextromethorphan and metoprolol (CYP2D6 substrates), respectively. These increases are due to celecoxib inhibition of the CYP2D6 substrate metabolism.

CYP2C19 inhibition
There is potential for celecoxib for celecoxib to inhibit CYP2C19 catalysed metabolism. The clinical significance of this in vitro finding is unknown. Examples of medicinal products which are metabolised by CYP2C19 are diazepam, citalopram and imipramine.

Methotrexate
In patients with rheumatoid arthritis celecoxib had no statistically significant effect on the pharmacokinetics (plasma or renal clearance) of methotrexate (in rheumatologic doses). However, adequate monitoring for methotrexate-related toxicity should be considered when combining these two medicinal products.

Lithium
Co-administration of celecoxib 200 mg twice daily with 450 mg twice daily of lithium may result

HVS 60 gsm, 372 mm x 200 mm, Empat kolom (dalam satu muka), Dua muka (bolak-balik), Satu bahasa (bahasa Inggris)   Print Out 75%

in a mean increase in Cmax of 16% and in AUC of 18% of lithium. Therefore, patients on lithium treatment should be closely monitored when celecoxib is introduced or withdrawn.

Oral contraceptives
Celecoxib had no clinically relevant effects on the pharmacokinetics of oral contraceptives (1 mg norethisterone /35 micrograms ethinylestradiol).

Glibenclamide/tolbutamide
Celecoxib does not affect the pharmacokinetics of tolbutamide (CYP2C9 substrate), or glibenclamide to a clinically relevant extent.

Effects of other medicinal products on celecoxib
CYP2C9 poor metabolisers
In individuals who are CYP2C9 poor metabolisers and demonstrate increased systemic exposure to celecoxib, concomitant treatment with CYP2C9 inhibitors such as fluconazole could result in further increases in celecoxib exposure. Such combinations should be avoided in known CYP2C9 poor metabolisers.

CYP2C9 inhibitors and inducers
Since celecoxib is predominantly metabolised by CYP2C9 it should be used at half the recommended dose in patients receiving fluconazole. Concomitant use of 200 mg single dose of celecoxib and 200 mg once daily of fluconazole, a potent CYP2C9 inhibitor, resulted in a mean increase in celecoxib Cmax of 60% and in AUC of 130%. Concomitant use of inducers of CYP2C9 such as rifampicin, carbamazepine and barbiturates may reduce plasma concentrations of celecoxib.

Ketoconazole and antacids
Ketoconazole or antacids have not been observed to affect the pharmacokinetics of celecoxib.

Paediatric population
Interaction studies have only been performed in adults.

FERTILITY, PREGNANCY AND LACTATION
Fertility
Based on the mechanism of action, the use of NSAIDs, including celecoxib, may delay or prevent rupture of ovarian follicles, which has been associated with reversible infertility in some women. In women who have difficulties conceiving or who are undergoing investigation of infertility, withdrawal of NSAIDs, including celecoxib, should be considered.

Pregnancy
There are no studies in pregnant women. Studies in animals have shown reproductive toxicity. The relevance of these data for humans is unknown. Celecoxib as with other drugs inhibiting prostaglandin synthesis, may cause uterine inertia and premature closure of the ductus arteriosus and should be avoided during the third semester of pregnancy. Celecoxib should be used during pregnancy only if the potential benefit to the mother justifies the potential risk to the foetus. Inhibition of prostaglandin synthesis might adversely affect pregnancy. If used during second or third trimester of pregnancy, NSAIDs may cause fetal renal dysfunction which may result in reduction of amniotic fluid volume or oligohydramnios in severe cases. Such effects may occur shortly after treatment initiation and are usually reversible. Pregnant women on celecoxib should be closely monitored for amniotic fluid volume.

Lactation
Administration of celecoxib to lactating women has shown very low transfer of celecoxib into breast milk. Because of the potential for adverse reactions in nursing infants from celecoxib, a decision should be made whether to discontinue nursing or to discontinue the drug, taking into account the expected benefit of the drug to the mother.

SIDE EFFECTS
Table 1: Adverse drug reactions in celecoxib clinical trials and surveillance experience (MedDRA preferred terms) [1,2]

Adverse Drug Reaction Frequency						
System Organ Class	**Very Common**	**Common**	**Uncommon**	**Rare**	**Very Rare**	**Frequency Not Known (cannot be estimated from available data)**
Infections and infestations		Sinusitis, upper respiratory tract infection, pharyngitis, urinary tract infection				
Blood and lymphatic system disorders			Anaemia	Leukopenia, Thrombocytopenia	Pancytopenia	
Immune system disorders		Hypersensitivity			Anaphylactic shock, anaphylactic reaction	
Metabolism and nutrition			Hyperkalaemia			
Psychiatric disorders		Insomnia	Anxiety, depression, fatigue	Confusional state, hallucinations		
Nervous system disorders		Dizziness, hypertonia, headache	Cerebral infarction, paraesthesia, somnolence	Ataxia, dysgeusia	Haemorrhage intracranial (including fatal intracranial haemorrhage), meningitis aseptic, epilepsy (including aggravated epilepsy), ageusia, anosmia	
Eye disorders			Vision blurred, conjunctivitis	Eye haemorrhage	Retinal artery occlusion, retinal vein occlusion	
Ear and labyrinth disorders			Tinnitus, hypoacusis			
Cardiac disorders		Myocardial infarction	Cardiac failure, palpitations, tachycardia	Arrhythmia		
Vascular disorders	Hypertension (including aggravated hypertension)			Pulmonary embolism, flushing	Vasculitis	
Respiratory, thoracic, and mediastinal disorders		Rhinitis, cough, dyspnoea	Brochopasm	Pneumonitis		
Gastrointestinal disorders		Nausea, abdominal pain, diarrhoea, dyspepsia, flatulence, vomiting, dysphagia	Constipation, gastritis, stomatitis, gastrointestinal inflammation (including aggravation of gastrointestinal inflammation), eructation	Gastrointestinal haemorrhage, duodenal ulcer, gastric ulcer, oesophageal ulcer, intestinal ulcer, large intestinal ulcer, intestinal perforation, oesophagitis, melaena, pancreatitis colitis		
Hepatobiliary disorders			Hepatic function abnormal, hepatic enzyme increased (including increased SGOT and SGPT)	Hepatitis	Hepatic failure (sometimes fatal or requiring liver transplant), hepatitis fulminant (some with fatal outcome), hepatic necrosis, cholestasis, hepatitis cholestatic, jaundice	
Skin and subcutaneous tissue disorders		Rash, pruritus (includes pruritus generalised)	Urticaria, ecchymosis	Angioedema, alopecia, photosensitivity	Dermatitis exfoliative, erythema multiforme, Stevens-Johnson syndrome, toxic epidermal necrolysis, drug reaction with eosinophilia and systemic symptoms (DRESS), acute generalised exanthematous pustulosis (AGEP), dermatitis bullous	
Musculoskeletal and connective tissue disorders		Arthralgia	Muscle spasms (leg cramps)		Myositis	
Renal and urinary disorders			Blood creatinine increased, blood urea increased	Renal failure acute, hyponatraemia	Tubulointerstitial nephritis, nephrotic syndrome, glomerulonephritis minimal lesion	
Reproductive system and breast disorders				Menstrual disorder		Infertility female (female fertility decreased)
General disorders and administrative site conditions		Influenza-like illness, oedema peripheral/fluid	Face oedema, chest pain			
Injury, poisoning and procedural complications		Injury (accidental injury)				

SYMPTOMS AND TREATMENT OF OVERDOSE
In the event of suspected overdose, appropriate supportive medical care should be provided e.g. by eliminating the gastric contents, clinical supervision and, if necessary, the institution of symptomatic treatment. Dialysis is unlikely to be an efficient method of medicinal product removal due to high protein binding.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINE
Patients who experience dizziness, vertigo or somnolence while taking Celexocib 200 mg Capsules should refrain from driving or operating machinery.

INSTRUCTIONS FOR USE/HANDLING, AND DISPOSAL
Celecoxib capsules can be taken with or without food.
Given the association between cardiovascular risk and exposure to COX-2 inhibitors, doctors are advised to use the lowest effective dose for the shortest possible duration of treatment.

STORAGE
Store below 30°C
Jauhi daripada kanak-kanak
Keep out of children's reach

SHELF LIFE
36 months

THERAPEUTICAL CODE
Pharmacotherapeutic classification (WHO's ATC classification) ATC code: M01AH01

PACKAGING
Box, 3 strips x 10 capsules ;
Box, 10 strips x 10 capsules
ON MEDICAL PRESCRIPTION ONLY

MANUFACTURED BY

NOVELL PHARMACEUTICAL LABORATORIES

PT. Novell Pharmaceutical Laboratories
Jl. Wanaherang No. 35, Tlajung Udik,
Gunung Putri, Bogor 16962 – Indonesia

PRODUCT REGISTRATION HOLDER
CELESTEMAL PHARMA SDN. BHD.
3 Jalan 19/1, 46300 Petaling Jaya
Selangor, Malaysia

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